

NUR258SCE Coronary Vascular Disorders Simulation Scenario

(Revised 12/17/2025 CA)

Client: Anna Marie Zerby

Discipline: Adult Health Nursing II

Expected Simulation Run Time: 5 hours.

Location: Joyce Simulation Center

Student Level: Advanced Med/tele

Location for Reflection: Joyce Briefing Center

File Name: NUR258SCE. Coronary Vascular
Disorders. NLN

Brief Description of Client

Hospital Location: Cardiac ICU

Shift: Day shift

Name: Anna Marie Zerby

Date of Birth: 1-18-19XX

Age: 44

Allergies: NKDA

Admit Weight: 220 lbs

Height: 5ft 6in

Pronouns: She/her

Marital Status: Married

Language: English

Sex: Female

Attending Provider/Team: John Mack

Primary Admitting Medical Diagnosis: Unstable Angina, post cardiac angiogram.

Past Medical History: Obesity, Anxiety, GERD, Hypercholesterolemia, Hypertension, Total Hysterectomy 2022, Right Carpel Tunnel surgery, 2020, current every-day smoker (1 pack a day).

History of Present Illness: Ms. Zerby is a 44-year-old female returning to her room from the cardiac catheterization lab after evaluation for unstable angina. The angiogram revealed four occluded coronary vessels, and no stents were placed. She has been admitted to the cardiac ICU and is scheduled for CABG surgery in the morning. She remains groggy from conscious sedation and has not yet been informed of the angiogram findings or the plan for surgery.

Notes for instructor: The client will present as anxious and concerned but cooperative and appropriate. Learners will be informed that the client is being transported from the cath lab to her inpatient room, and the scenario will begin with the client as a new admission.

Simulation Design Template (revised February 2023)

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NUR258 Course Outcomes

1. Use critical thinking, clinical reasoning, and clinical judgement to prioritize comprehensive, individualized, and holistic care of the adult client. (CO1)
2. Select appropriate priorities for nursing assessment and interventions when delivering safe nursing care for adults. (CO2)
3. Deliver evidence-based holistic nursing care for adult clients with health alterations. (CO3)
4. Evaluate outcomes of nursing care of adult clients with health alterations. (CO4)

Simulation Scenario Objectives

1. Use critical thinking to recognize cues for Acute Coronary Syndrome. (CO1)
2. Based on recognized cues, perform appropriate interventions for a client having Acute Coronary Syndrome. (CO1)
3. Reassess a client after taking action and performing interventions for a client experiencing Acute Coronary Syndrome. (CO1)

Faculty Reference

The Healthcare Simulation Standards of Best Practice™

<https://www.inacsl.org/healthcare-simulation-standards>

Setting/Environment

Cardiac ICU

Equipment/Supplies for Coronary Vascular Disorder: Anna Marie Zerby

Simulated Patient/Manikin(s) Needed: HAL

Recommended Mode for Simulator: preprogramed manual advance

Other Props & Moulage:

Equipment Attached to Manikin/Simulated Patient: • ID band for Anna Maria Zerby • Female genitalia • Female Wig • IV in place and working. (saline locked) Additional Moulage • TR band placed on Right wrist. • IV Left Forearm • Purse with Xanax pill bottle and cigarettes.	Equipment Available in Room: • Equipment to take vitals: Pulse oximeter, telemetry cables (3 lead), thermometer. • O2 delivery device: nasal cannula and simple mask. • Toothette or toothbrush • Alaris IV pump • Blood collection supplies: vacutainer, gauze, tape, flushes, butterfly syringe, & lab tubes. • Bedpan/urinal • Black bin to deposit used supplies. • Emesis basin • Water cup and drinking straw.
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- Suction tubing and suction canister.
- Grip socks
- Incentive spirometer

Beginning Scenario Vitals Monitor programming:

HR: 88 **BP:** 132/68 **Temp:** 37.0
RR: 16 **SpO2:** 96%

All Medications and fluids needed for scenario (and quantity of medication)

Oral Medications

- Atorvastatin 20 mg PO (1 tab)
- Acetaminophen 650 mg (2 tabs)
- Aspirin 81 mg PO (4 tabs)
- Metoprolol 25 mg PO (1 tab)
- Nitroglycerin 0.4 mg SL tabs (3)

Green: Need in SimServe

Yellow: in the room, running

IV Injections

- Lorazepam 1 mg, IV push (1 vial)
- Ondansetron 4 mg, IV push (1 vial)

Transdermal

- Nicotine Patch 21mg/24hr Extended Release

Roles

Students	Simulation Staff
Lead Nurse	Nursing Instructor & SOS/Tech Team acting as following: • Physician • Charge Nurse • Pharmacy • Radiology • Cardiac/EKG Tech • Phlebotomy/Lab • Operator • Patient

Guidelines/Information Related to Roles

All- Employ the TeamSTEPPS Principles of leadership, team structure, communications, situation monitoring & mutual support. Monitor patients' condition and intervene as appropriate.

Lead Nurse- Responsible for PT outcome & team performance. Conduct briefing assign tasks & set common goals & objectives. Patient assessment. Direct the team in an efficient & effective manner to improve the patient's condition. Communicate with the team and MD.

Pre-briefing/Briefing

Students and instructor will discuss the following in pre-briefing:

1. Priorities based on client cues.
2. Physical assessment and client cues.
3. Expected and concerning lab results.
4. Expected and concerning diagnostic results.
5. Forming a hypothesis and making recommendations based on client cues.
6. Expected medications for client.
7. Stable vs unstable angina
8. Nuclear stress test results
9. Complications from cardiac cauterization
10. CABG Education

Student Shift Report: **verbally** given to students by instructor.

(Please do not give the students a physical copy of this.)

Client Name Anna Marie Zerby	Code Status Full Code	Allergies NKDA	DOB 1-18-19XX Age 44	Rm # TBD	MD John Mack
Vital Signs (last set done on night shift) HR 86 O2 97% RA BP 127/72 Temp 36.8 RR 14			Health History: Obesity, Anxiety, GERD, Hypercholesterolemia, Hypertension, Total Hysterectomy in 2022, Right Carpel Tunnel surgery 2020, current everyday smoker (1 pack a day). Home Medications: Atenolol, Xanax, Pepcid AC		
Admitting Medical Diagnosis: Unstable Angina					
History of Present Illness: Ms. Zerby is a 44-year-old female being transported back to her room following a cardiac angiogram performed in the cardiac catheterization lab for unstable angina. She was taken to the cath lab for possible angioplasty and stent placement; however, stenting was not feasible due to findings of four-vessel coronary artery disease and poor coronary anatomy. She has been scheduled for coronary artery bypass grafting (CABG) tomorrow morning. Ms. Zerby is still groggy from the procedure but tolerated it without complications, and her vital signs have remained stable. Ms. Zerby initially presented to her primary healthcare provider the previous day with complaints of shortness of breath occurring with minimal activity and increased stress. Her symptoms were relieved with rest. She denied chest, arm, or jaw pain but reported diaphoresis. Due to these findings, her provider referred her for an outpatient nuclear stress test earlier today, which was positive for ST-segment depression, shortness of breath, and a drop in blood pressure.					
Neuro Wakes up when talked to, but still groggy. Oriented x4.			Cardiology Normal Sinus Rhythm, S1, S2		
Respiratory Clear, equal			GI Active in all quadrants.		
Skin/Wounds Clean, dry, intact R Radial access site TR band in place			GU Voids, continent. Has not voided since before procedure.		
Drains/IVs 18 g, L forearm Saline locked			Diagnostics Orders are pending.		
Pain Denies pain			Fall Risk Low		
Psycho/Social Married. Husband at work.			Diet NPO at midnight.		

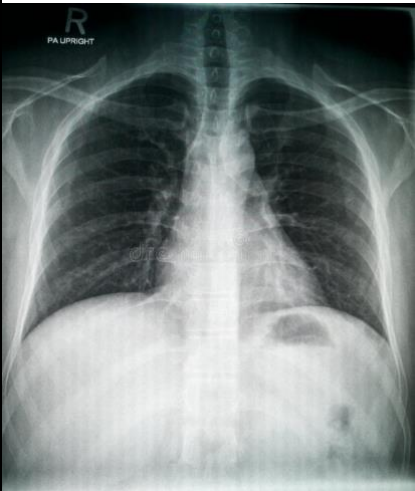
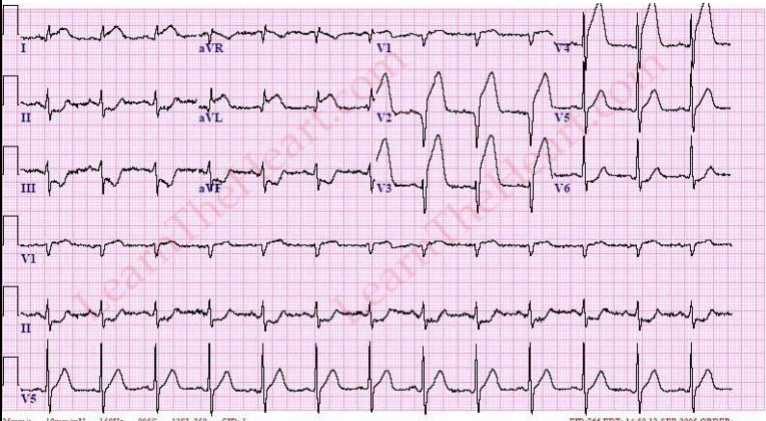
ATI: INITIAL ORDERS (Everything listed below will already be visible to the student in ATI when the scenario is started.)

<u>Medications</u>	<u>Labs</u>	<u>Nursing/Misc. orders</u>
- Nicotine Patch 21mg/24hr extended-release PRN Home Medications (listed in ATI under NOTES) - Atenolol 50mg PO daily - Xanax 10mg PO daily - Pepcid AC 10mg PO daily	- CMP, once now - CBC, once now - Troponin every hour until levels have peaked. EKG results from nuclear stress test: ST-depression, acute ischemia	- Admit to Cardiac ICU - Continuous telemetry - Continuous vital signs. - Pulses and neurovascular checks every 15 min for one hour to right arm, then every 30 min for 2 hours, then hourly until stable. - Encourage oral fluids, then NPO after midnight - Saline lock IV while taking IV fluids. - Supplemental Oxygen as needed to maintain SPO2 great than 92%.

ATI: AVAILABLE SCENARIO ORDERS (these will all be available to push out in the scenario)

<u>Medications</u>	<u>Labs</u>	<u>Nursing/Misc. orders</u>
- Lorazepam 1 mg IV push, PRN every 4 hours for anxiety. - Ondansetron 4 mg IV push, PRN for nausea and vomiting. Chest Pain Orders - Aspirin 324 mg, chewable, once for chest pain PRN. - Nitroglycerin 0.4 mg SL, every five min x3 PRN for chest pain. Hold for inferior MI or SBP less than 110.	Pre-operative CABG Orders - Type/screen and crossmatch for 2 units of PRBC's. - Pre-operative education for CABG. - LABS to order in AM: CBC, CMP, Cardiac enzymes Chest Pain Orders - Troponin, STAT 12-Lead EGK, STAT - Chest X-ray to rule out aortic aneurysm.	NONE

Scenario Labs/Diagnostics labs to be pushed out during the scenario.

CMP & CBC	Chemistry/ Metabolic <table><tr><td>Na</td><td>140</td><td>135-145</td></tr><tr><td>K</td><td>4.2</td><td>3.5-5</td></tr><tr><td>Cl</td><td>102</td><td>98-106</td></tr><tr><td>CO2</td><td>26</td><td>22-29</td></tr><tr><td>Creatinine</td><td>0.9</td><td>0.6-1.2</td></tr><tr><td>BUN</td><td>18</td><td>10-20</td></tr><tr><td>Glucose</td><td>95</td><td>60-100</td></tr></table> CBC <table><tr><td>WBC</td><td>9</td><td>5-10</td></tr><tr><td>HCT</td><td>12.2</td><td>14-18</td></tr><tr><td>HGB</td><td>36</td><td>42-52</td></tr><tr><td>Platelets</td><td>250</td><td>150-400</td></tr></table>	Na	140	135-145	K	4.2	3.5-5	Cl	102	98-106	CO2	26	22-29	Creatinine	0.9	0.6-1.2	BUN	18	10-20	Glucose	95	60-100	WBC	9	5-10	HCT	12.2	14-18	HGB	36	42-52	Platelets	250	150-400
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Cardiac Enzymes	Cardiac Enzymes <table><tr><td>CPK</td><td>86</td><td>30-170</td></tr><tr><td>CK-MB</td><td>4.3</td><td>3-5%</td></tr><tr><td>Troponin</td><td>0.15</td><td>0-0.5</td></tr></table>	CPK	86	30-170	CK-MB	4.3	3-5%	Troponin	0.15	0-0.5																								
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Chest X-Ray	Chest X-ray Within normal limits. 																																	
12 Lead EKG	12 Lead EKG STEMI anterior wall 																																	

Scenario Progression Outline

Patient Name: Anna Marie Zerby

Date of Birth: 01/18/19xx

MD: John Mack

Manikin/SP Actions	Expected Nursing Interventions	MD Orders	Student Cues
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Vitals HR: 88 BP: 132/68 RR: 16 SpO2: 96% RA Temp: 37.0 Pain: none • Head: WNL • Neuro: Sleepy, oriented X4 • Cardiac: NSR, S1 and S2 • Resp: Clear, equal • Abdomen: WNL • GI/GU: Nauseated and vomiting. • Skin: R radial puncture site, TR band in place. • Pulses: +3, equal • Capillary Refill: less than 3	Student Actions before calling MD. • Place telemetry leads. • Call for labs. • Perform head-to-toe assessment. • Asses R Radial site/TR band.	MD orders • Lorazepam 1 mg, IV push. • Ondansetron 4 mg, IV push. Pre-operative CABG Orders • Type/screen and crossmatch for 2 units of PRBC's. • Pre-operative education for CABG. • LABS to order in AM: CBC, CMP, Cardiac enzymes	Student Cues • Client is groggy but will become more awake as the student performs assessment. • Client asks about how the stent placement went. • Client will be anxious when she finds out that stents were not placed and that she will have a CABG in the morning. Patient is emotional and concerned, but not hysterical. • Client will ask for her purse with Xanax in it. Can also ask for a cigarette **Anxiety and nausea will improve with medication.
Vitals Change HR: 132 (PVCs 30) BP: 164/98 RR: 24 SpO2: 89% RA Temp: 37.0 Pain: 8/10 chest pain, radiates to left jaw, back, and L arm.	Student Actions before calling MD. • Assess chest pain. • Reassess vitals. Scenario objectives: Use critical thinking to recognize cues for Acute Coronary Syndrome. (CO 1) ☐ Irregular rhythm ☐ Chest pain ☐ Anxiety ☐ Nausea Based on recognized cues, perform appropriate interventions for a client having Acute Coronary Syndrome. (CO2) ☐ Administer Aspirin ☐ Administer Nitro Reassess a client after taking action and performing interventions for a client experiencing Acute Coronary Syndrome. (CO 4). ☐ Chest pain ☐ Blood pressure	MD orders Chest Pain Orders • Aspirin 324 mg, chewable, once for chest pain PRN. • Nitroglycerin 0.4 mg SL, every five min x3 PRN for chest pain. Hold for inferior MI or SBP less than 110. • Troponin, STAT • 12-Lead EKG, STAT • Chest X-ray to rule out aortic aneurysm. After EKG Results • Prep client to go to OR. MD will come talk to her immediately.	Student Cues • Client is experiencing extreme chest pain. • Worried she is having a heart attack. • Chest pain improves with Nitro. Optional V/S After administering nitro -> lower BP to 100/60 Can allow students to administer second dose of nitro.

Debriefing/Guided Reflection

Debriefing Phase	Debriefing Questions for Consideration
Reactions/ Defuse	How did you feel throughout the simulation experience?
	Give a brief summary of this patient and what happened in the simulation.
	What were the main problems that you identified?
Analysis/ Discovery	Discuss the knowledge guiding your thinking surrounding these main problems.
	What were the key assessment and interventions for this patient?
	Discuss how you identified these key assessments and interventions.
	Discuss the information resources you used to assess this patient. How did this guide your care planning?
	Discuss the clinical manifestations evidenced during your assessment. How would you explain these manifestations?
	Explain the nursing management considerations for this patient. Discuss the knowledge guiding your thinking.
	What information and information management tools did you use to monitor this patient's outcomes? Explain your thinking.
	How did you communicate with the patient?
	What specific issues would you want to take into consideration to provide for this patient's unique care needs?
	Discuss the safety issues you considered when implementing care for this patient.
	What measures did you implement to ensure safe patient care?
	What other members of the care team should you consider important to achieving good care outcomes?
	How would you assess the quality of care provided?
	What could you do improve the quality of care for this patient?
Summary/ Application	If you were able to do this again, how would you handle the situation differently?
	What did you learn from this experience?
	How will you apply what you learned today to your clinical practice?
	Is there anything else you would like to discuss?

Coronary Vascular Disorders Anna Marie Zerby

Section	Content
Scenario Name	Coronary Vascular Disorders – Acute Coronary Syndrome / STEMI (Anna Marie Zerby)
Setting	Cardiac ICU post-cardiac angiogram patient with severe coronary artery disease awaiting CABG
Primary Learning Focus	Recognition and management of acute coronary syndrome after cardiac catheterization with emphasis on chest pain assessment ischemia recognition medication safety and reassessment after nitroglycerin
Priority Hypothesis	Acute coronary syndrome progressing to anterior wall STEMI in a patient with known four-vessel coronary artery disease following recent angiogram
Secondary Hypotheses	Post-procedural complication at radial access site anxiety increasing myocardial oxygen demand nausea and vomiting related to ischemia or procedural recovery
Key Supporting Cues	ST depression on prior stress test four-vessel coronary artery disease sudden severe chest pain radiating to jaw back and left arm nausea anxiety PVCs rising HR hypertension followed by optional BP drop after nitroglycerin STEMI on 12 lead EKG
Red Flags / Escalation Triggers	New chest pain radiating to jaw arm or back HR greater than 130 PVCs increasing BP or hemodynamic instability SpO2 less than 92 percent ischemic EKG changes persistent nausea anxiety and diaphoresis
Expected Clinical Progression if Missed	Unstable angina → myocardial ischemia → STEMI → dysrhythmias cardiogenic shock cardiac arrest and need for emergent intervention
Common Student Errors	Focusing on anxiety or nausea without recognizing ischemia delaying chest pain assessment forgetting aspirin or nitroglycerin not reassessing blood pressure after nitro overlooking the significance of four-vessel disease or EKG changes
Clinical Judgment Focus	Recognizing the transition from post-procedure recovery to active ischemia linking symptoms to ACS implementing chest pain protocol promptly and reassessing response to treatment
Real-World Translation	This patient would require immediate telemetry provider notification 12 lead EKG aspirin nitroglycerin if appropriate and urgent movement toward definitive intervention such as CABG or cath lab level management
Instructor Teaching Emphasis	Students should recognize that severe coronary disease plus new radiating chest pain plus ischemic EKG findings represents a cardiac emergency and requires immediate action and reassessment

Coronary Vascular Disorders Anna Marie Zerby

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching/Debrief Focus	High Risk	Common Missed Items
Scenario Overview	Post-cardiac angiogram	Recent invasive cardiac procedure with risk for ischemic and access-site complications	Important	Creates need for focused cardiovascular and neurovascular monitoring	Missed bleeding ischemia or post-procedure complication	Assess site pulses rhythm symptoms and vitals	Ongoing	B	Post-procedure patients can deteriorate quickly		No
Scenario Overview	Four-vessel coronary artery disease	Severe multivessel disease with major risk for myocardial ischemia and infarction	Immediate	Indicates very high cardiac risk	Delay in recognizing the seriousness of new symptoms	Recognize severe disease and monitor closely	Ongoing	A	Severity of disease changes the urgency of symptoms	★	Yes
Scenario Overview	Awaiting CABG in the morning	Patient is already identified as needing surgical revascularization	Important	Confirms advanced coronary disease	False reassurance if patient seems stable	Prepare patient and anticipate escalation	Ongoing	B	Planned CABG means the disease is significant		No
Scenario Overview	Primary scenario focus = acute coronary syndrome progressing to STEMI	Core teaching point of scenario	Immediate	Life-threatening ischemia requires rapid intervention	Myocardial damage dysrhythmia shock arrest	Recognize ACS and implement chest pain protocol	Ongoing	A + R	This is not just an anxiety or nausea scenario	★	Yes
Baseline / Shift Report	Full code	Background for escalation planning	Relevant	Important background only	Minimal	Know code status	N/A	S	Background awareness only		No
Baseline / Shift Report	NKDA	Medication safety background	Relevant	Supports safe administration of emergent meds	Medication error	Verify allergies before meds	N/A	B	Always verify allergies		No
Baseline / Shift Report	Age 44	Younger adult with severe premature CAD	Relevant	Context only but notable severity for age	Underestimating cardiac risk	Routine age-aware care	N/A	B	ACS can occur in younger adults too		No
Baseline / Shift Report	MD John Mack	Provider for escalation and orders	Relevant	Needed for communication	Delay in escalation	Know who to call	N/A	R	Communication readiness		No
Baseline / Shift Report	HR 86 baseline	Stable baseline heart rate	Relevant	Useful comparison point for later increase	Miss trend toward instability	Trend HR	Ongoing	A	Trends matter		No
Baseline / Shift Report	O2 97 percent RA baseline	Normal oxygenation at start	Relevant	Useful comparison for later decline	Miss oxygenation change	Trend SpO2	Ongoing	A	Know the baseline before decline		No
Baseline / Shift Report	BP 127/72 baseline	Stable baseline perfusion	Relevant	Useful comparison point	Miss hemodynamic changes	Trend BP	Ongoing	A	Trend awareness		No

Coronary Vascular Disorders Anna Marie Zerby

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching/Debrief Focus	High Risk	Common Missed Items
Baseline / Shift Report	RR 14 baseline	Normal respiratory status at start	Relevant	Useful comparison point	Miss increased work of breathing	Trend RR	Ongoing	A	Trends matter		No
Baseline / Shift Report	Obesity	Cardiovascular risk factor	Relevant	Contributes to disease burden	Underestimating risk	Include in clinical picture	N/A	B	Risk factors help explain presentation		No
Baseline / Shift Report	Anxiety	May increase sympathetic stimulation and myocardial oxygen demand	Important	Can worsen ischemia but is not the primary problem	Over-focusing on anxiety and missing ACS	Support patient while assessing for ischemia	Reassess symptoms	B	Do not label chest pain as anxiety too early	✓	Yes
Baseline / Shift Report	GERD history	Potential non-cardiac cause of chest discomfort in other settings	Relevant	Could confuse assessment but not primary here	Misattributing ischemic pain to reflux	Consider but prioritize cardiac causes	N/A	B	Cardiac pain must be ruled out first		Yes
Baseline / Shift Report	Hypercholesterolemia	Strong CAD risk factor	Relevant	Supports severe vascular disease	Underestimating disease burden	Include in cardiac risk profile	N/A	B	Risk factors matter		No
Baseline / Shift Report	Hypertension	Cardiac risk factor and hemodynamic context	Relevant	Important with nitro and chest pain management	Medication misjudgment or poor BP interpretation	Monitor BP closely	Ongoing	B	BP history shapes medication decisions		No
Baseline / Shift Report	Current smoker 1 pack/day	Major CAD risk factor and nicotine withdrawal issue	Relevant	Contributes to ACS risk	Underestimating severity or distracting with smoking request	Provide education later and prioritize ACS	N/A	B	Smoking is relevant but not the immediate priority during chest pain		Yes
Baseline / Shift Report	Home meds Atenolol Xanax Pepcid AC	Background meds showing BP anxiety and GI management	Relevant	Helpful context but not current priority	Minimal	Review for history only	N/A	B	Home meds provide context		No
Baseline / Shift Report	Admitting diagnosis unstable angina	High-risk coronary syndrome already present	Immediate	Patient already has symptomatic ischemia history	Delay in recognizing worsening ACS	Maintain high suspicion and monitor	Ongoing	A	Unstable angina can progress quickly	★	Yes
Baseline / Shift Report	Groggy post procedure	Residual sedation may affect assessment and communication	Important	Can mask symptom severity	Missed deterioration or delayed questioning	Assess LOC and continue monitoring	Frequent reassessment	A	Sedation can hide important cues	✓	Yes
Baseline / Shift Report	Oriented x4	Baseline mentation	Relevant	Useful comparison point	Miss later change in LOC	Trend neuro status	Ongoing	B	Know normal baseline		No
Baseline / Shift Report	Normal sinus rhythm baseline	Stable rhythm at first	Relevant	Important comparison for later PVCs and ischemia	Miss rhythm deterioration	Trend telemetry	Ongoing	A	NSR can become unstable quickly		No

Coronary Vascular Disorders Anna Marie Zerby

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching/Debrief Focus	High Risk	Common Missed Items
Baseline / Shift Report	Respiratory clear and equal	No baseline pulmonary finding	Relevant	Useful comparison point	Miss changes or over-focus on lungs	Trend respiratory status	Ongoing	B	Clear lungs do not rule out ACS		No
Baseline / Shift Report	GI active bowel sounds	GI system stable at baseline	Relevant	Not current priority	Minimal	Routine	N/A	B	Low priority finding		No
Baseline / Shift Report	Radial access site with TR band	Risk for bleeding or neurovascular compromise post cath	Immediate	Post-procedure complication risk	Hemorrhage ischemia to limb	Assess site pulses and TR band	Ongoing	A + R	Post-cath access site checks are essential	★	Yes
Baseline / Shift Report	Has not voided since before procedure	May reflect limited intake or post-procedure delay but needs monitoring	Important	Potential perfusion or post-procedural concern	Delayed recognition of retention or low perfusion	Monitor voiding status and fluids	Ongoing	B	Low output or no void can matter after procedure	✓	Yes
Baseline / Shift Report	18 gauge left forearm saline lock	IV access available for meds	Important	Needed for IV medications and escalation	Delayed treatment if not checked	Verify IV patency	Reassess site	B	Always verify access		No
Baseline / Shift Report	Diagnostics pending	Workup still in progress	Important	Current status may change with new results	Delay in treatment if not followed up	Call for labs and obtain diagnostics	Reassess with results	B	Pending data must be actively followed		No
Baseline / Shift Report	Pain denied at baseline	No chest pain initially	Relevant	Useful comparison point for later onset	Miss progression from stable to unstable	Reassess pain frequently	Ongoing	A	Absence of pain now does not lower vigilance		Yes
Baseline / Shift Report	Low fall risk baseline	Baseline safety status	Relevant	May change if chest pain or hypotension develops	Injury from sudden weakness or nitro-related BP drop	Reassess fall risk when symptoms change	Ongoing	B	Fall risk can change with instability	✓	Yes
Baseline / Shift Report	Married husband at work	Psychosocial support context	Relevant	Background only	Minimal	Include in support and teaching as appropriate	N/A	B	Holistic care		No
Baseline / Shift Report	NPO at midnight	Pre-op preparation order	Important	Supports planned CABG	Unsafe intake if surgery imminent	Maintain order and update if status changes	Ongoing	B	Orders may escalate as condition worsens		No
Pre-Scenario Orders	Nicotine patch PRN	Comfort measure for nicotine withdrawal	Relevant	Not a priority during acute ischemia	Distraction from urgent needs	Address after stabilization	N/A	B	Do not prioritize comfort requests during ACS		Yes
Pre-Scenario Orders	CMP once now	Part of cardiac workup	Important	Provides metabolic baseline	Miss contributors to instability	Obtain and review	Reassess with results	B	Current labs matter in ACS workup		No

Coronary Vascular Disorders Anna Marie Zerby

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching/Debrief Focus	High Risk	Common Missed Items
Pre-Scenario Orders	CBC once now	Part of workup	Important	Rules out contributing issues and supports baseline	Ongoing diagnostic uncertainty	Obtain and review	Reassess with results	B	Workup is part of nursing follow-through		No
Pre-Scenario Orders	Troponin every hour until peaked	Tracks myocardial injury	Immediate	Essential ACS trending data	Missed MI progression	Obtain and trend serial troponins	Reassess with results	A + R	Serial troponins matter more than one isolated value	★	Yes
Pre-Scenario Orders	EKG from nuclear stress test showed ST depression acute ischemia	Strong pre-existing ischemic clue	Immediate	Patient is high risk for ACS progression	Delayed recognition of evolving MI	Keep ischemia high on differential	Ongoing	A	Prior diagnostic results change urgency	★	Yes
Pre-Scenario Orders	Continuous telemetry	Essential cardiac monitoring	Immediate	Needed for PVCs and ischemic rhythm changes	Missed dysrhythmia or evolving instability	Ensure telemetry in place and interpreted	Ongoing	A	Telemetry must be read not just applied	★	Yes
Pre-Scenario Orders	Continuous vital signs	High-risk monitoring order	Important	Signals patient requires close observation	Missed hemodynamic deterioration	Monitor continuously and trend	Ongoing	A	Continuous monitoring fits patient risk level	✓	Yes
Pre-Scenario Orders	Pulses and neurovascular checks to right arm	Important post-cath access monitoring	Immediate	Detects bleeding or arterial compromise	Loss of perfusion or hematoma	Assess site pulses cap refill and band	Ongoing	A + R	Right radial checks are part of safety	★	Yes
Pre-Scenario Orders	Encourage oral fluids then NPO after midnight	Supports renal clearance and pre-op preparation	Relevant	Context only unless status changes	Minimal	Follow order when stable	N/A	B	Orders may be interrupted by instability		No
Pre-Scenario Orders	Saline lock IV while taking oral fluids	Maintains IV access without continuous infusion	Important	Allows rapid medication administration if needed	Delayed treatment	Verify saline lock patency	Reassess IV access	B	Saline lock still needs assessment		No
Pre-Scenario Orders	Oxygen as needed to maintain SpO2 >92%	Clear oxygen parameter	Important	Needed once symptoms and SpO2 worsen	Delayed support for ischemia/hypoxia	Apply oxygen when indicated	Reassess saturation	A	Know ordered oxygen targets		No
Available Orders	Lorazepam IV PRN for anxiety	May reduce anxiety and sympathetic drive	Relevant	Helpful but not priority over ACS treatment	Masking ischemia if misused	Treat anxiety after cardiac assessment	Reassess anxiety	B	Do not treat anxiety before ruling out worsening ACS		Yes

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching/Debrief Focus	High Risk	Common Missed Items
Available Orders	Ondansetron IV PRN	Treats nausea and vomiting	Important	Supportive care and comfort	Aspiration or worsening distress	Administer when indicated	Reassess nausea	B	Supportive care still matters		No
Available Orders	Aspirin 324 mg chewable once PRN chest pain	Core ACS treatment	Immediate	Reduces platelet aggregation and improves outcomes	Missed timely ACS therapy	Administer promptly when chest pain starts	Reassess pain and status	A + R	Aspirin is a first-line chest pain intervention	★	Yes
Available Orders	Nitroglycerin 0.4 mg SL q5 min x3 PRN chest pain	Core ACS symptom treatment	Immediate	Improves coronary perfusion and decreases pain	Persistent ischemia or unsafe administration if contraindicated	Assess pain BP and administer if safe	Reassess pain and BP after each dose	A + R	Nitro requires BP reassessment and contraindication awareness	★	Yes
Available Orders	Hold nitro for inferior MI or SBP less than 110	Medication safety parameter	Immediate	Prevents dangerous hypotension or harm	Severe hypotension	Check EKG location and BP before dose	Reassess BP after nitro	A + R	Students must know when not to give nitro	★	Yes
Available Orders	Type and screen crossmatch 2 units PRBCs	Pre-op CABG preparation	Important	Supports surgery readiness	Delay in OR readiness	Ensure order completion	Verify pending status	R	Preparation continues even during instability		No
Available Orders	Pre-op CABG education	Important patient education need	Important	Supports informed consent and preparedness	Increased anxiety or poor understanding	Provide or reinforce education when appropriate	Reassess understanding	B	Teaching still matters but timing matters too		No
Available Orders	AM labs CBC CMP cardiac enzymes	Pre-op and ischemia monitoring	Important	Supports ongoing cardiac evaluation	Delayed OR readiness or missed changes	Ensure future orders understood	N/A	R	Anticipate what comes next		No
Available Orders	Troponin stat for chest pain	Urgent cardiac injury assessment	Immediate	Part of acute chest pain workup	Missed evolving MI	Obtain promptly	Reassess with result	A + R	Chest pain requires renewed workup	★	Yes
Available Orders	12 lead EKG stat for chest pain	Confirms ischemic changes and STEMI	Immediate	Determines next level of intervention	Delay in diagnosis and OR prep	Obtain immediately and interpret	Reassess with result	A + R	Do not delay EKG in chest pain	★	Yes
Available Orders	Chest x-ray to rule out aortic aneurysm	Part of differential workup	Important	Rules out alternate emergency causes	Missed alternate diagnosis	Obtain promptly if ordered	Review results	B	Differential diagnosis still matters		No
Scenario Results	CMP normal overall	No major metabolic abnormality driving presentation	Relevant	Helps narrow cause toward ACS	Over-focusing on normal labs	Use results to support cardiac hypothesis	N/A	B	Normal labs help narrow the differential		No

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching/Debrief Focus	High Risk	Common Missed Items
Scenario Results	CBC values listed but appear internally mislabeled	Lab section suggests no major acute hematologic emergency driving chest pain	Relevant	Not the primary problem here	Distraction from ischemia	Focus on clinically meaningful abnormalities	N/A	B	Do not let documentation oddities distract from patient cues		Yes
Scenario Results	Troponin 0.15	Within listed reference but should be interpreted with symptoms and EKG	Important	Single value does not rule out active ischemia	False reassurance	Trend serial troponins	Reassess with repeat values	B	One troponin does not end the workup	✓	Yes
Scenario Results	Chest X-ray WNL	No acute chest x-ray cause identified	Relevant	Supports ischemia-focused plan	Minimal	Continue ACS pathway	N/A	B	Normal chest x-ray does not rule out MI		No
Scenario Results	12 lead EKG = STEMI anterior wall	Definitive ischemic emergency finding	Immediate	Confirms active myocardial infarction	Massive myocardial damage delay to intervention	Activate urgent response and prep for OR/definitive care	Reassess after actions	A + R	This is the turning point of the scenario	★	Yes
Initial Progression	HR 88	Stable starting HR	Relevant	Useful comparison	Minimal	Trend	Ongoing	A	Know starting point		No
Initial Progression	BP 132/68	Stable starting BP	Relevant	Useful comparison and nitro safety baseline	Miss later drop	Trend BP	Ongoing	A	Baseline BP matters before nitro		No
Initial Progression	SpO2 96 percent RA	Stable initial oxygenation	Relevant	Useful comparison	Miss later decline	Trend SpO2	Ongoing	A	Know the baseline		No
Initial Progression	No pain initially	Initially asymptomatic for chest pain	Relevant	Useful comparison to later acute event	Miss progression	Reassess pain	Ongoing	A	Symptoms can change quickly		No
Initial Progression	Sleepy but oriented x4	Residual sedation but intact orientation	Important	Can mask evolving symptoms	Miss subtle changes	Assess LOC and responsiveness	Reassess neuro	B	Sedation can complicate assessment		No
Initial Progression	Nauseated and vomiting	Post-procedure or ischemia-related symptom	Important	Can be an early ischemic symptom	Aspiration delayed care	Administer ondansetron and assess for ischemia	Reassess nausea	B	Nausea can be cardiac		Yes
Initial Progression	R radial puncture site with TR band	Post-cath access site needing assessment	Immediate	Potential bleeding or perfusion issue	Loss of perfusion or hematoma	Assess site pulses cap refill and band	Reassess neurovascular checks	A + R	Do not forget the access site		Yes
Initial Progression	Pulses plus 3 equal	Initial perfusion to extremities intact	Relevant	Baseline neurovascular status	Miss later change	Trend checks	Ongoing	B	Know baseline perfusion		No
Initial Progression	Cap refill less than 3	Normal initial perfusion	Relevant	Baseline only	Minimal	Trend	Ongoing	B	Baseline comparison		No

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching/Debrief Focus	High Risk	Common Missed Items
Initial Progression	Student should place telemetry leads	Required monitoring step	Immediate	Needed before deterioration	Missed rhythm changes	Apply telemetry promptly	Ensure monitoring active	R	Setup matters in high-risk patients	✓	Yes
Initial Progression	Student should call for labs	Required workup step	Important	Pending data supports decision-making	Delayed treatment plan	Obtain labs and results	Review results	R	Follow through on pending diagnostics		No
Initial Progression	Student should perform head-to-toe assessment	Core initial assessment requirement	Immediate	Finds access-site and procedure-related concerns	Miss important cues	Perform full assessment	Reassess when patient changes	A	Assessment first		No
Initial Progression	Student should assess R radial site and TR band	Essential post-cath safety check	Immediate	High-risk complication area	Missed bleeding or ischemia	Assess site and pulses	Reassess per protocol	A + R	Site checks are not optional		Yes
Initial Progression	Patient asks how stent placement went	Cue to discuss bad news and anxiety	Important	Emotional distress may escalate symptoms	Poor communication or worsening anxiety	Respond honestly within scope and support patient	Reassess emotional state	B	Communication and support matter		No
Initial Progression	Patient becomes anxious when told no stents and CABG in morning	Emotional trigger increasing myocardial oxygen demand	Important	Anxiety can worsen ischemia	Increased chest pain or hemodynamic strain	Provide support and PRN medication as ordered	Reassess anxiety	B	Anxiety can worsen the cardiac picture		Yes
Initial Progression	Patient asks for purse with Xanax or cigarette	Comfort-seeking behavior and nicotine dependence cue	Relevant	Not priority during instability	Distraction from priorities	Maintain focus on medical urgency	N/A	B	Do not let comfort requests derail assessment		Yes
Initial Progression	Lorazepam and ondansetron improve anxiety and nausea	Shows early supportive measures can reduce distress	Relevant	Helpful but does not treat ischemia if it occurs later	False reassurance	Continue monitoring after symptom relief	Reassess anxiety and nausea	B	Supportive relief does not equal cardiac stability		Yes
Chest Pain Progression	HR 132 with PVCs 30	Acute sympathetic surge and ectopy during ischemia	Immediate	Signals worsening cardiac instability	Dysrhythmia worsening ischemia arrest	Assess immediately and place on high alert	Reassess HR rhythm	A + R	PVCs during chest pain raise concern for ischemia severity	★	Yes
Chest Pain Progression	BP 164/98	Acute hypertensive response with chest pain	Important	Increases myocardial oxygen demand and affects nitro decisions	Worsening ischemia	Reassess BP and treat ACS	Ongoing	A	Interpret BP in context of pain and nitro		No
Chest Pain Progression	RR 24	Tachypnea with pain and distress	Important	Supports acute event	Miss worsening distress	Trend RR	Ongoing	B	Tachypnea can accompany ACS		No

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching/Debrief Focus	High Risk	Common Missed Items
Chest Pain Progression	SpO2 89 percent RA	Hypoxia during chest pain event	Immediate	Below ordered target in active ACS	Worsening ischemia and organ hypoxia	Apply oxygen	Reassess SpO2	A + R	Oxygen demand rises during ischemia	★	Yes
Chest Pain Progression	Chest pain 8 out of 10 radiating to jaw back and left arm	Classic ischemic chest pain	Immediate	Strong ACS cue requiring immediate protocol	Delayed treatment myocardial damage	Assess pain characteristics and act promptly	Reassess pain after aspirin and nitro	A + R	Radiating pain is a major ACS clue	★	Yes
Chest Pain Progression	Extreme chest pain with fear of heart attack	Symptomatic and emotionally distressed patient	Immediate	Patient recognizes seriousness of symptoms	Delay in action if dismissed	Take symptoms seriously and reassure while treating	Reassess distress and pain	A	Patient statements are important clinical cues		Yes
Chest Pain Progression	Assess chest pain before calling MD	Required focused assessment step	Immediate	Needed for correct intervention and SBAR	Weak escalation and delayed meds	Assess quality location radiation severity	Ongoing	A	Focused pain assessment matters		No
Chest Pain Progression	Reassess vitals before calling MD	Needed to guide treatment and nitro safety	Immediate	BP and SpO2 determine action	Unsafe nitro or poor communication	Obtain current vitals	Ongoing	A	Assessment precedes medication		No
Chest Pain Orders	Aspirin 324 mg chewable now	Immediate antiplatelet ACS treatment	Immediate	Improves outcomes and reduces clot propagation	Delayed ACS treatment	Administer promptly	Reassess pain and status	A + R	Aspirin should happen quickly in chest pain	★	Yes
Chest Pain Orders	Nitroglycerin 0.4 mg SL every 5 min x3 PRN	Acute ischemia symptom treatment	Immediate	Relieves pain and decreases workload	Worsening ischemia or unsafe administration if contraindicated	Check BP then administer if appropriate	Reassess pain and BP after each dose	A + R	Give nitro safely and reassess every time	★	Yes
Chest Pain Orders	Troponin stat	Updates myocardial injury status	Important	Part of repeat ACS workup	Miss evolving infarction	Obtain and trend	Reassess with result	B	Serial markers matter		No
Chest Pain Orders	12 lead EKG stat	Confirms STEMI and next steps	Immediate	Determines definitive response	Delay to OR or intervention	Obtain immediately	Interpret result	A + R	EKG changes direct the next move	★	Yes
Chest Pain Orders	Chest x-ray to rule out aortic aneurysm	Supports differential diagnosis	Important	Rules out other catastrophic causes	Miss alternative diagnosis	Obtain if ordered	Review result	B	Keep differential in mind while treating ACS		No

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	S B A R	Teaching/Debrief Focus	High Risk	Common Missed Items
After Results	Prep client to go to OR	Next-level intervention after STEMI identification	Immediate	Definitive care is urgent	Delay in reperfusion or surgical treatment	Prepare patient rapidly and maintain monitoring	Ongoing	R	Once STEMI confirmed bedside care must move toward definitive intervention	★	Yes
Optional Response	After nitro BP may decrease to 100/60	Expected medication effect requiring caution	Immediate	Nitro can lower BP significantly	Hypotension syncope unsafe repeat dosing	Reassess BP before repeating nitro	Trend vitals	A + R	Reassessment determines whether more nitro is safe	★	Yes
Expected Reassessment	Chest pain improves with nitro	Desired treatment response	Immediate	Indicates symptom relief but not resolution of underlying cause	Miss ongoing ischemia if reassessment is skipped	Ask directly about pain after each dose	Reassess pain score and radiation	A + R	Improvement must be measured not assumed	★	Yes
Expected Reassessment	Blood pressure after nitro	Medication safety endpoint	Immediate	Determines whether another dose is safe	Hypotension from repeated nitro	Measure BP after each dose	Trend BP	A + R	Nitro requires BP reassessment every time	★	Yes
Scenario-Wide Teaching Point	Severe CAD plus new radiating chest pain equals emergency	Core learning point	Immediate	Symptoms and history together define urgency	Delayed recognition and treatment	Act immediately and escalate	N/A	A	Context plus symptoms creates urgency	★	Yes
Scenario-Wide Teaching Point	Reassess after every ACS intervention	Core nursing principle	Immediate	Need to know whether aspirin nitro oxygen changed status	Miss failed response or medication harm	Repeat pain vitals rhythm oxygenation after each action	Ongoing	A + R	Every intervention needs reassessment	★	Yes
Scenario-Wide Teaching Point	Do not treat this as anxiety alone	Common reasoning error	Important	Anxiety is present but not primary	Missed STEMI	Rule out ischemia first	N/A	B	Cardiac emergencies can present with anxiety and nausea	✓	Yes
Scenario-Wide Teaching Point	Know nitro safety parameters	Medication safety principle	Immediate	Inferior MI and hypotension are contraindication concerns	Medication-induced harm	Check BP and EKG context before repeating dose	N/A	A	Students must know when to hold nitro	★	Yes

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Section	Debrief Question	Instructor Prompt
Reaction / Defusing	How did you feel throughout the simulation experience?	Encourage students to identify their emotional response before moving into analysis.
Reaction / Defusing	Give a brief summary of this patient and what happened in the simulation.	Guide students to summarize the transition from post-cath recovery to acute coronary syndrome and STEMI recognition.
Reaction / Defusing	What were the main problems that you identified?	Prompt students to identify acute coronary syndrome chest pain ischemic changes and the need for urgent intervention as the priority problems.
Analysis / Discovery	Discuss the knowledge guiding your thinking surrounding these main problems.	Encourage students to connect symptoms history and EKG findings to myocardial ischemia.
Analysis / Discovery	What were the key assessments and interventions for this patient?	Guide students toward chest pain assessment telemetry EKG aspirin nitroglycerin oxygen if needed and reassessment.
Analysis / Discovery	Discuss how you identified these key assessments and interventions.	Prompt students to explain why the symptoms and history were concerning for ACS rather than anxiety alone.
Analysis / Discovery	Discuss the information resources you used to assess this patient. How did this guide your care planning?	Encourage students to include the shift report stress test result telemetry 12 lead EKG labs and bedside assessment.
Analysis / Discovery	Discuss the clinical manifestations evidenced during your assessment. How would you explain these manifestations?	Guide students toward radiating chest pain nausea PVCs anxiety hypertension and hypoxia as manifestations of acute ischemia.
Analysis / Discovery	Explain the nursing management considerations for this patient. Discuss the knowledge guiding your thinking.	Prompt students to prioritize rapid ACS treatment medication safety right radial site checks and OR preparation.
Analysis / Discovery	What information and information management tools did you use to monitor this patient's outcomes? Explain your thinking.	Encourage students to identify telemetry 12 lead EKG serial troponins vital signs pain reassessment and neurovascular checks.

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Section	Debrief Question	Instructor Prompt
Analysis / Discovery	How did you communicate with the patient?	Guide students to reflect on calm supportive communication while the patient was frightened and asking about her outcome.
Analysis / Discovery	What specific issues would you want to take into consideration to provide for this patient's unique care needs?	Prompt students to consider anxiety smoking history severe CAD procedural recovery and medication safety with nitroglycerin.
Analysis / Discovery	Discuss the safety issues you considered when implementing care for this patient.	Encourage students to focus on nitro blood pressure effects access-site safety continuous monitoring and rapid escalation.
Analysis / Discovery	What measures did you implement to ensure safe patient care?	Guide students toward checking BP before nitro applying telemetry obtaining EKG promptly and preparing for definitive care.
Analysis / Discovery	What other members of the care team should you consider important to achieving good care outcomes?	Prompt students to include provider cardiology OR team pharmacy and telemetry support.
Analysis / Discovery	How would you assess the quality of care provided?	Encourage evaluation of timeliness prioritization communication and reassessment.
Analysis / Discovery	What could you do improve the quality of care for this patient?	Prompt students to focus on earlier recognition stronger chest pain assessment and faster transition from bedside care to definitive intervention.
Summary / Application	If you were able to do this again how would you handle the situation differently?	Encourage students to identify one or two specific behavior changes.
Summary / Application	What did you learn from this experience?	Guide students toward a clear takeaway about ACS recognition and medication reassessment.
Summary / Application	How will you apply what you learned today to your clinical practice?	Prompt students to connect chest pain recognition aspirin nitro use and reassessment to future bedside care.
Summary / Application	Is there anything else you would like to discuss?	Use this as an invitation for unresolved questions or reflection.

Coronary Vascular Disorders (ACS)

Primary Learning Focus

Recognition and management of unstable angina following cardiac catheterization while preparing the patient for CABG.

Critical Cues

- ☐ Groin/cath site assessment
- ☐ Chest pain
- ☐ Hemodynamic changes
- ☐ Anxiety regarding diagnosis

Expected Priorities

- ☐ Focused cardiovascular assessment
- ☐ Recognition of ischemia
- ☐ Prioritize patient safety after catheterization

Expected Interventions

- ☐ Assess cath insertion site
- ☐ Monitor circulation/perfusion
- ☐ Administer ordered cardiac medications
- ☐ Patient education and reassurance

Expected Reassessment

- ☐ Chest pain
- ☐ Distal pulses
- ☐ Hemodynamics

Common Student Errors

- Forgetting cath site assessment
- Missing bleeding complications
- Focusing only on pain
- Incomplete patient education

CJMM-ALIGNED SIMULATION DEBRIEFING QUICK REFERENCE GUIDE

This guide provides flexible reflection prompts aligned with the NCSBN Clinical Judgment Measurement Model (CJMM). Select, adapt, and sequence questions based on simulation objectives, student performance, and learner needs. Use the prompts as a cognitive aid to support discussion and reflection, not as a required script or checklist.

1. Recognize Cues

- What patient findings or changes in condition stood out to you?
- Which assessment findings were most important?
- What changes suggested that the patient's condition was improving or worsening?
- Were there any important cues that were initially missed or overlooked?

2. Analyze Cues

- What did the assessment findings suggest was happening with the patient?
- How were the important cues related to one another?
- How did the patient's pathophysiology help explain the findings?
- What additional information would have helped you better understand the patient's condition?

3. Prioritize Hypotheses

- What did you believe was the patient's highest-priority problem?
- Which patient problem or concern required attention first, and why?
- What was the greatest immediate risk to the patient?
- How did you decide which problem was most urgent?

4. Generate Solutions

- What nursing interventions did you consider for the patient's priority problem?
- What outcome did you expect from those interventions?
- What other interventions or approaches could have been considered?
- What additional resources, information, or team members could have helped guide the plan of care?

5. Take Action

- What action did you take first, and why?
- How did the patient's condition influence the order of your interventions?
- Were there any actions that should have been taken earlier or differently?
- How did communication and teamwork affect the care provided?

6. Evaluate Outcomes

- How did you determine whether the patient responded to the interventions?
- What findings needed to be reassessed after the interventions?
- Did the patient's response require a change in the plan of care?
- What would you continue to monitor or reassess if care of this patient continued?

7. Reflection and Future Clinical Application

- What went well, and what challenged you the most?
- What would you do differently if you encountered a similar situation again?
- How will you apply what you learned from this experience to future patient care?